

A School-Age Child in Your Clinic

For family doctors, nurses & health professionals — caring for an autistic primary-school child (roughly ages 6–11).

The one idea

Autism, through **monotropism**, means a child's attention locks into a few deep, intense "tunnels" at a time — vivid inside, faint outside, and costly to switch. School is often the first real stress test: noisy, visually busy and unpredictable, it drains the attention that should go to everything else. Many children **hold it together at school and collapse afterwards** — including in your waiting room. Working with the tunnel, not against it, is the key.

What you may see — and what's really going on

You may see...	What's often happening
"Fine at school," meltdowns or shutdowns at home or in the clinic	The child masked all day; home — and your surgery — is where the mask finally drops
Stomach aches, headaches, "won't eat"; no clear organic cause	High sensory and social load shows up through the body; also common: GI and sleep co-occurrence
Difficulty starting, switching or stopping; seems "lazy" or defiant	Autistic inertia — changing mental track costs real energy, not willpower
Suddenly "lost" skills they had last year	Often burnout / a shift in attentional resources under overwhelm, not true regression
Takes things literally; needs extra time to answer	Single-channel, literal communication + processing time

Try this

- **Send a pre-visit form** (the AASPIRE *AHAT*, autismandhealth.org) and ask the parent what helps; welcome headphones, sunglasses and a comfort object.
- **Book the quietest slot**, dim the lights, lower beeps; allow a trusted adult in the room.
- **Explain each step before doing it**, in short literal sentences; let the child stim or hold an interest object.
- **Ask about school load and recovery**, not just symptoms — "How long to recover after a school day?" flags overwhelm.
- **Co-create a simple health routine** (same room, same sequence) for repeat visits.

Avoid this

- Treating masked composure as “they’re fine,” or a shutdown as refusal to engage.
- Long verbal instructions, surprise procedures, or “be brave!” phrasing.
- Pushing harder when skills have dropped — the evidence-based response to burnout is **reduce demands and restore rest and interests**.

When to get more support

Watch for **autistic burnout** (chronic exhaustion, loss of skills, reduced tolerance to stimulus) — it is *distinct from depression* and from ordinary stress, and is worsened by masking. Screen for the common co-occurrences: **anxiety, ADHD (AuDHD), sleep and GI problems**. Ask directly about school attendance and bullying. Signpost SENCO/SEND support and autism-led parent groups.

If the child is a girl

Girls this age **camouflage heavily** and are frequently missed or misdiagnosed (anxiety, “shy”). Take parental concern seriously even when teachers see a quiet, compliant child — the cost is being paid out of sight.

About this guide

AI-assisted, derived from this project's research drafts — the **Monotropism** brief and the **Lifespan Practitioner & Health-Care guide** (Parts 3.2 & 4). Uses identity-first language. **Informational — not medical advice or a diagnostic tool**. Fit it to the individual child.

Key sources. Murray, Lesser & Lawson (2005); Murray, F. (2019, *Monotropism at School*; 2023); Dwyer (2025); Edgar (2024, regression/burnout); Raymaker et al. (2020); AASPIRE AHAT; Kelly Mahler (2024). Full citations are in the source drafts.